Document Control Report

Title Contingency Plan for Increased Demand on Critical Care Services Policy			
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Version	Date Issued	Status	Comment / Changes / Approval
1.0	12 Jan 2012	Draft	
1.1	Feb 14	Revision	Updated to meet current service
2.0	Mar 14	Final	To be Published on Bob

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Document Class Policy	**Target Audience** All Trust and Sodexho staff involved in the escort and transfer of patients between wards, departments, hospitals internal to, and external to the Trust.
Distribution List Senior Management	**Distribution Method** Intranet site
Superseded Documents Contingency Plan for Increased Demand for ITU Services	

Issue Date	Review Date	Review Cycle
March 2014	March 2017	Three years

Consulted with the following stakeholders: (list all) • Consultant and senior nursing staff Critical Care, Accident and Emergency, Resuscitation teams, theatres and Recovery. • Senior Nurses and Ward Managers • Hotel Services Coordinator • Compliance Manager	**Contact responsible for implementation and monitoring compliance:** Senior Nurse/Matron Critical Care- Formal Audit of inter hospital transfers undertaken by Torbay Hospital. **Education/ training will be provided by:** • Clinical Educator and Senior Nursing team Critical Care.

	• Senior Nursing Team Recovery
Approval and Review Process • Patient Safety Committee	**Ratified by Trust Board?** No
Local Archive Reference G:\ **Local Path** Critical Care Policies\ **Filename** Contingency Plan for Increased Demand on Critical Care Services Policy V2.0 07Mar14	
Policy categories for Trust's internal website Nursing, harmonised,	**Tags for Trust's internal website**

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1 Introduction

This document sets out Northern Devon Healthcare NHS Trust's system for managing increased demand over capacity on critical care services. It provides a robust framework to ensure a consistent approach across the whole organisation, and supports our statutory duties as set out in the NHS Constitution.

This policy is intended for all critically ill patients who require critical care services within the Trust and is applicable on all days of the week both days and nights.

2 Purpose

The purpose of this document is to ensure that all critically ill patients shall be managed safely within a safe and appropriate environment, without compromising their condition whilst maintaining high standards of care. This policy has been developed in response to the recognised risk to patients, personnel and equipment used when demand for critical care services exceeds the capacity within the critical care unit as it stands currently.
This policy excludes those patients who are requiring specialist care from identified centres.

3 Definitions

3.1 Critical Care Unit

A designated area specifically equipped and staffed to manage the care of critically ill patients.

3.2 Recovery Unit

A designated area specifically equipped and staffed to manage patients following planned or emergency surgery.

3.3 Critically ill patient

Critically ill patients in this policy are defined as requiring a level of care greater than that normally provided on a standard ward.

3.3.1 Levels of Care

Level 0: Requires hospitalisation needs can be met through normal ward care.

Level 1: (Step Down) Recently discharged from a higher level of care, in need of additional monitoring/clinical interventions, clinical input or advice.

Level 2: (High Dependency) Patients needing pre-operative optimisation extended post operative care, stepping down from Level 3.

Level 3: (Intensive Care) Patients receiving advanced respiratory support alone, advanced cardiovascular support alone or a minimum of 2 organs supported.

3.4 Nurse to patient ratio

The recommended nurse to patient ratio is 1:1 for Level 3 and 2:1 for Level 2 patients in critical care (ICS) and 1:1 for recovery patients until they meet

discharge criteria maintaining own airway and cardiovascularly stable.(AAGB! 2003)

3.5 Critical Care Team

The critical care team can consist of one or more members of the following doctors, nursing, operating department assistants or nurses who are suitably trained and competent to care for critically ill patients within the critical care unit or recovery area either independently or with supervision.

3.6 Patient Groups

Patients in the following groups maybe cared for under the critical care units responsibility and in collaboration with an identified consultant:

3.6.1 Medical patient

An adult patient who is admitted to a critical care environment who requires ongoing specialist critical care treatment and interventions which are not provided outside a critical care setting.

3.6.2 Surgical patient

An adult patient who has undergone either planned or emergency surgery who requires ongoing specialist critical care treatment or interventions which are not provided outside a critical care setting.

3.6.3 Paediatric patient

A patient who is under the age of 18 (19years of age in the case of patients with learning difficulties or complex needs) who is admitted to a critical care environment who requires ongoing specialist critical care treatment and interventions which are not provided outside a critical care setting.

4 Responsibilities

4.1 Role of Director of Nursing

The Director of Nursing is responsible for ensuring there are robust processes in place for the safe care of patients requiring critical care when there is pressure on critical care bed demand and patients are to be cared for outside the normal critical care environment and ensuring that all senior management are aware of this policy.

4.2 Role of Duty Critical Care Consultant

The Duty Intensive Care Consultant is directly responsible for ensuring:

- Intensive Care Society Standards and Guidelines for the care of critically ill patients (3rd Edition 2011)" are followed at all times in order to maintain high standards of care.
- The most appropriate setting is identified for ongoing care of an admission i.e. ICU, Recovery or Emergency Department.
- The clinical care of all critically ill patients is managed in collaboration with the patients identified admitting consultant when appropriate.
- Identifying any patient who is medically fit for transfer back to a general ward.

- Identifying the most appropriate/stable critically ill patient who can be managed safely in the Recovery area. if there is no bed immediately available on ICU the patient waiting admission will be accommodated in the Recovery area until a bed is made available under the care of the Duty Intensive care Consultant and medical team.
- Ensuring a clear medical management plan is in place for all the critically ill patients whilst they are being cared for the Recovery area.
- Ensure appropriate medical cover is available to oversee the care of a critically ill patient whilst they are accommodated in the Recovery area.
- Working collaboratively with the Nurse Coordinators from ICU and Recovery to identify, risk assess and plan for any potential issues to the patients, staff or the Trust as a whole.
- Identifying any critically ill patients who maybe appropriate to transfer to another hospital within our network or to an outside specialist centre.
- Ensuring the escalation plan is initiated in accordance with Trust and national policy via the Clinical Site Managers (Bleep 500) where non clinical transfers for capacity reasons are required (ICS Recommendation 7.4.3).
- Ensuring effective communication between areas and teams is maintained at all times as adjustments may be required to supporting areas working schedules.
- Ensuring any issues/risks are handed over to the following duty intensive care consultant, medical team and coordinators and escalated to CSM or Duty Manager.

4.3 Role of Critical Care Doctor

The doctor is responsible for ensuring:

- The ongoing medical management of the critically ill patients under the care of the critical care team.
- The direct management of a ventilated patient in the Recovery area and the doctor is expected to stay in the recovery area with the patient unless a clinically competent critical care nurse is caring for the patient.
- The discharge/transfer documentation is completed and a handover is given to receiving ward medical teams for any patient to be transferred to the ward or another critical care provider in the network.
- Ensuring a comprehensive handover is given to the next medical team on shift.

4.4 Role of Critical Care Co-ordinator

The nurse co-ordinator in charge is responsible for:

- Ensuring Intensive Care Society Standards and Guidelines for care of critically ill adult (3rd Edition 2011) are followed in order to maintain high standards of care.
- Undertaking a risk assessment and collaborate with the Duty Intensive Care Consultant and Recovery Coordinator to initiate the Contingency Plan for increased demand on Critical Care Services.
- Informing the Senior Nurse/Matron Critical Care, Clinical Site Mangers and Duty Manager that the demand on capacity exceeds the beds available.
- Directing the nursing team to transfer any patient medically fit for the ward back to an appropriate ward for ongoing care.
- Co-ordinating the transfer of any patient identified by the ICU consultant as appropriate to the Recovery area for ongoing management.
- Working collaboratively with the recovery co-ordinator to establish any staffing issues to ensure the patients are able to receive the same level and standard of care.
- Contacting the Emergency Bed Service (EBS) to establish the overall position within our own Critical Care Network in the South West and where beds are available if demand on capacity increases further. .
- Initiating the staffing escalation cascade to increase staffing by at least one Registered Nurse to cover and support the unexpected increase in demand in the Recovery area. Consider requesting support from within own nursing establishment i.e. senior nurse/ matron, clinical educator or specialist nursing teams i.e., Pain Team, Resuscitation Team, Nursing Clinical Support Team for Level 2 patients, Emergency Department, NHSP or Agency.
- Working collaboratively with the Duty Intensive Care Consultant and Recovery coordinator to identify, risk assess and plan for any potential issues to the patients, staff or the Trust as a whole.
- Ensuring effective communication between areas and teams is maintained
- Ensuring any issues/risks are handed over to the follow on nursing team, duty intensive care consultant, medical team and Recovery coordinator and escalated to CSM or Duty Manager.

4.5 Role of the Recovery Coordinator

The Recovery Area Coordinator is responsible for ensuring

- Ensuring Association of Anaesthetists of Great Britain and Ireland Recommendations for immediate post anaesthetic recovery (2007) and recommendations for Standards of monitoring during anaesthesia and are followed in order to maintain high standards of care.
- Undertaking a risk assessment and collaborate with the Duty Intensive Care Consultant and ICU coordinator to initiate the Contingency Plan for increased demand on Critical Care Services.
- Informing the Operating Department Manager, Clinical Site Mangers and Duty Manager the impact increasing ICU capacity to the recovery

area will have on the day to day working within the Operating Department.

- Directing the transfer of any patients suitably recovered ftom there anaesthetic back to the ward for ongoing care.
- The recovery area is prepared for the transfer of any patient identified by the ICU consultant as appropriate to the Recovery area for admission or ongoing management.
- Working collaboratively with the ICU coordinator to establish any staffing issues to ensure the patients are able to receive the same level and standard of care..
- Considering the implications on the Operating Department Service if patients are required to remain on the Recovery area for extended periods of time up to and greater than 24hours.
- Working collaboratively with the Duty Intensive Care Consultant and ICU coordinator to identify, risk assess and plan for any potential issues to the patients, staff or the Trust as a whole.
- Ensuring effective communication between areas and teams is maintained.
- Ensuring any issues/risks are handed over to the follow on nursing team, duty intensive care consultant, medical team and ICU coordinator and escalated to CSM or Duty Manager.

4.5 Role of the Critical Care Senior Nurse/Matron

The Senior Nurse/Matron is responsible for ensuring:

- Senior Management teams are aware of the situation within the Critical Care Unit
- Provides clinical support to the Critical Care team and Recovery team as required.
- Support ICU and Recovery teams to provide safe care.
- Review staffing and ensure strategies are in place to increase staffing levels where appropriate.
- Provide effective communication and support between the Critical Care and Recovery teams.

4.6 Role of the Critical Care Nurse/ Recovery Nurse/Practitioner

The Registered Nurse/Recovery Nurse/Practitioner is responsible for ensuring:

- They are suitably trained and competent to undertake the care of the critically ill patient.
- They communicate effectively any concerns regarding their level of experience/ competence to the ICU coordinator and/or Recovery coordinator to enable appropriate allocation of support.

- The assessment, planning, delivery and evaluation of quality critical care and maintaining observation recordings and documentation throughout.
- Effective communication between the medical, nursing and recovery team.
- The patient and their next of kin / carer have been informed of the need to transfer to the recovery area.
- Escalate any concerns to the ICU coordinator or CSM Bleep 500.
- Ensure a comprehensive handover is given to the next shift nurse/practitioner.

4.7 Role of Clinical Site Managers (CSM Bleep 500)

The Clinical Site Manager is responsible for ensuring:

- The Duty Manager is made aware of the increased demand on capacity on critical care beds, the immediate plan to accommodate this and the impact on theatre services.
- CSM to identify ward beds for any patients medically fit for discharge.
- Collaborative working and communication with both the ICU and Recovery co-ordinators takes place to ensure every effort to maintain normal working within both areas.
- Escalate any staffing issues to the Duty Manager and request Agency cover when required and release shifts when approved.

4.8 Role of Duty Manager (Blp 065)

The Duty Manager is responsible for ensuring:

- The Executive Director on call is made aware of the increased demand on capacity on critical care beds, the immediate plan to accommodate this and the impact on theatre services.
- Appropriate support and leadership is provided to the clinical and CSM teams to maintain effective working and quality safe care.
- Escalate in accordance with Trusts Major Incident Policy as required.

4.9 Role of Supernumery Registered Nurse or Student Nurse

Supernumery Registered Nurses or Student Nurses should never undertake the care of a critically ill patient in the Recovery area unsupervised by an a suitably trained or experienced intensive care nurse.

5 Critical Care Overflow

The Main Theatres Recovery area has been identified as an appropriate safe area where critical care patients can receive ongoing support and management for a short period whilst the demand for critical care services is increased above capacity.

Implementing this will ensure that:

The risk associated with increased demand is reduced.by utilising the recovery staff in an area where medical and nursing support from the critical care unit can be accessed directly.

The critical care consultant and the critical care nurse coordinator will identify the most appropriate patient to be cared for within the recovery area

This will ensure that:

- Appropriately competent personnel and equipment are involved in the management of patients outside the critical care environment. This may include involving the resuscitation and acute pain teams to support and manage the patients care and staff outside the critical care unit
- Patients are assessed and care is planned and evaluated on an ongoing basis without compromise to their condition in the recovery area.
- Open communication channels are maintained between the two areas managing the critical care patients to allow prioritisation of care and provide safe environments .
- Staff within both areas are fully supported and engaged in collaborative team working.
- The risk of harm to our patients through infections or pressure ulcers is reduced through continuity of care within our own site.
- At all times, consideration is given to the preservation of a patient's and their relative's privacy and dignity and to their physical, psychological and emotional needs.
- In the event there is a further increase in the demand on the identified capacity the critical care consultant and nurse coordinator will contact the Emergency Bed Service (EBS) and identify available beds within our critical care network in the South West.
- CSM, Duty Manager and Executive Director on call must be kept up to date on the critical care bed situation at each point of change.to enable Trust Escalation/Major Incident plans to be initiated if required.

There should be no differentiation in the safety and quality of care critical patients receive within the identified critical care settings.

6 Training requirements

Recovery staff have transferable skills which can be used in the care of critically ill patients however their experience is varied and limited and thus require ongoing support and supervision from a competent experienced critical care nurse. As a team their level of competence and transferable skills

include airway management and ventilation during an anaesthetic,, administration of medications and recording of respiratory, cardiovascular observations and fluid management. Training around more specific drug administration such as inotropes is required.

There is an agreement with senior management to increase ICU establishment by 1 registered nurse when demand exceeds capacity on ICU and patients are being cared for the recovery area. This allows for flexibility and ability to move an intensive care nurse to the recovery area or provide support to recovery staff. On an informal basis staff are encouraged to work across both areas when own workloads allow.

Formal mandatory training is available and accessed through workforce development.

7 Monitoring Compliance With and the Effectiveness of the Policy

Standards/ Key Performance Indicators

Key performance indicators comprise:

- Routine monitoring of incidents.
- Routine monitoring of complaints and patient feedback.
- Monthly audit of ICU/HDU patient stays in recovery area

Process for Monitoring Compliance and Effectiveness

Monitoring compliance with this policy will be the responsibility of the Senior Nurse/Matron Critical Care and Recovery Manager. This will be undertaken by

- Monitoring of incident report forms, and the identification of individual incidents, trends and themes will be discussed and / or action planning at the Critical Care Staff meeting and at the Patient Safety Operational Committee.
- Review of Monthly Audit data compiled by Recovery

Where non-compliance is identified, support and advice will be provided to improve practice.

Senior Nurse Critical Care and Recovery Manager will be responsible for monitoring and reporting to the Patient Safety Operational Group.

The Patient Safety Operational Group will develop an action plan to improve compliance and ensure improvements in performance occur, following review of incident trends and themes.

Action plans will be implemented by the Head of Quality and Safety to ensure learning takes place.

The Patient Safety Operational Group will monitor progress of the action plan on a monthly basis and exceptions will be reported via this group to the Safer Care Delivery Committee. Identified risks related to the non-compliance with this policy through audit will be registered on the Trust Risk Register system by the Risk Co-ordinator.

8 Equality Impact Assessment

Group	Positive Impact	Negative Impact	No Impact	Comment
Age			X	
Disability			X	
Gender			X	
Gender Reassignment			X	
Human Rights (rights to privacy, dignity, liberty and non-degrading treatment), marriage and civil partnership			X	
Pregnancy			X	
Maternity and Breastfeeding			X	
Race (ethnic origin)			X	
Religion (or belief)			X	
Sexual Orientation			X	

9 References

Department of Health. Comprehensive Critical Care: a review of adult critical acre services. Department of Health: London May 2000

http://www.ics.ac.uk/intensive_care_professional standards4302.pdf

www.ncod.org.uk/pdf/2003/03sum.pdf

www.ics.ac.uk/downloads/Standards/RCN20%20Critical%20care.pdf

The Anaesthetic Association of Anaesthetists of Great Britain and Ireland : Immediate post anaesthetic recovery: London.

The Anaesthetic Association of Anaesthetists of Great Britain and Ireland 2007 Recommendations for Standards of monitoring during anaesthesia and recovery 4th edition London

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9 Trust Policy referred to

- Bed Management Policy
- Nurse Staff Management policy
- Incident Management Policy
- Transfer of Adult Critical Care Patients Policy